2.13 Acid–Base Imbalances*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I cannot get enough air." **Objective** • ABG: pH 7.28, $PaCO_2$ 58 mmHg • Drowsy • SpO_2 88%	**ACTUAL** Impaired gas exchange related to alveolar hypoventilation as evidenced by respiratory acidosis, hypoxemia and drowsiness.	• Blood gases moving toward normal within 12 hours.	• Follow ABG, SpO_2 and consciousness as ordered. • Sit him upright to improve ventilation. • Give controlled oxygen to the prescribed target. • Withhold sedatives unless specifically ordered. • Prepare for non-invasive ventilation if pH or $PaCO_2$ worsens.	• Considered met when pH and $PaCO_2$ improve and he is alert.
Subjective • Feels nauseated and weak **Objective** • Deep, rapid breathing • ABG: pH 7.25, HCO_3^- 14 mmol/L	**ACTUAL** Ineffective breathing pattern related to respiratory compensation for metabolic acidosis as evidenced by deep, rapid respirations.	• Respiratory rate 12–20/min as the acidosis corrects.	• Observe rate and pattern of breathing. • Treat the cause as prescribed, for example insulin and fluids. • Give bicarbonate only on specific order. • Watch potassium closely.	• Goal reached when breathing pattern normalizes.
Subjective *No specific complaint* **Objective** • Abnormal pH • Hypoxemia	**RISK** Risk for acute confusion as evidenced by risk factors: acid–base imbalance and hypoxemia.	• Remains oriented.	• Check orientation every 4 hours. • Keep safety measures in place. • Report any change in mental state.	• Met if orientation is maintained.

* **References for this care plan:** [35], [37], [106], [108], [128]. Full entries are listed in the References section of the CPMN book.